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Critical Care – Time is Of Essence

There is nothing more challenging in medicine than caring for patients who face imminent death due to catastrophic illness. It requires quick assessment, advanced technology and prompt action to save these patients. Critical

care is fast emerging as a specialty in its own right in our country, some 15 years after its recognition in the developed world.

Modern medicine has witnessed an explosion of knowledge, technological sophistication, and new skills in managing complex illnesses. Today we are able to treat cancer, perform organ transplantation and other demanding surgical operations and salvage more patients with serious infections, injuries and organ failure. All this has been possible through the phenomenal growth of critical care medicine. As medicine is becoming increasingly modernized in India, there is a parallel development of the specialty.

Critical care involves immediate protection of the patient's breathing and blood circulation, two functions without which death would ensue within a few minutes. Then it requires capabilities to rapidly diagnose the problem through advanced imaging and laboratory facilities. The patient's progress is assessed minute-to-minute using monitoring devices. The patient may need artificial respiratory support with ventilators, pressor drugs to maintain his blood pressure and on occasion's dialysis to take over failing kidney function. Special training and expertise is required by the Intensivists directing the complex management. Capable medical and nursing teams continuously monitor the patient and respond to rapidly changing physiology. All this requires teamwork, organizational skills, and appropriate infrastructure.

The critical care physician works in an atmosphere of high technology medical interventions. Yet he has to learn the art of using his tools in a humane manner. He has to learn to minimize costs of care, communicate effectively with and support distraught families of patients, and to deliver effective but safe care. Through the maze of machines and tools he must remind himself that he is treating a person, not a set of abnormal physiological parameters. Since death is common in the critical care unit the critical care team needs to learn how to

manage death humanely without unduly prolonging the dying process. The field of critical care medicine demands the very best from the physician.

— Dr R. K. Mani

Director-Pulmonology and Critical care

Sudden Death

Sudden death from cardiac arrest is a major health problem that's received much less publicity than heart attack. Sudden cardiac death (also called sudden arrest) is death resulting from an abrupt loss of heart function (cardiac arrest). The time and mode of death are unexpected. It occurs within minutes after symptoms appear. The most common underlying reason for patients to die suddenly from cardiac arrest is blockages in the arteries. Most of the cardiac arrests that lead to sudden death occur when the electrical impulses in the diseased heart become rapid (ventricular tachycardia) or chaotic (ventricular fibrillation) or both. This irregular heart rhythm (arrhythmia) causes the heart to suddenly stop beating.

Brain death and permanent death start to occur in just four to six minutes after someone experiences cardiac arrest. Cardiac arrest is reversible in most victims if it's treated within a few minutes with an electric shock to the heart to restore a normal heartbeat. This process is called defibrillation. A victim's chances of survival are reduced by 7 to 10 percent with every minute that passes without CPR and defibrillation.

These episodes can now be treated by implanting devices called defibrillators. These are placed under the skin like pacemakers and treat abnormal rhythms if and when they arise.

— Dr. Balbir Singh (Cardiologist)

Director Physiology and Cath - Lab

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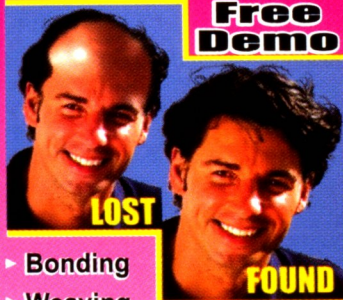
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